

Supplementary Materials
Diagnostic performance of quantitative measures
from FDG-PET/CT, FEC-PET/CT and
DW-MRI in the detection of lymph node
metastases in endometrial and cervical cancer.

Ben King
b.g.king@sms.ed.ac.uk

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Characteristic	n = 99
Median age (range), years	63.5 (24-80)
Median BMI (range), kg/m ²	28 (18-50)
Histology	
Cervical (n = 33)	
Adenocarcinoma	11 (33%)
Adenosquamous	6 (18%)
Squamous cell	15 (45%)
Unknown	1 (3%)
Endometrial (n = 66)	
Endometrioid	35 (53%)
Serous/clear cell	26 (39%)
Other	2 (3%)
Mixed (2 or more)	3 (5%)
Surgical Procedure	
Laparoscopic	42 (42%)
Robotic	31 (31%)
Open	26 (26%)
Lymphadenectomy	
Left pelvic	67 (68%)
Right pelvic	73 (74%)
Para-aortic	26 (26%)
FIGO stage (on MRI)	
Cervical (n = 33)	
IB1	29 (88%)
IB2	1 (3%)
IIA1	2 (6%)
IIB	1 (3%)
Endometrial (n = 66)	
IA	18 (27%)
IB	16 (24%)
II	9 (14%)
IIIA	3 (5%)
IIIC	14 (21%)
IVB	5 (8%)
Unknown	1 (2%)

Table 1: Demographics of patients enrolled onto the MAPPING study and eligible for analysis by meeting the primary reference standard of surgically confirmed nodal histology.

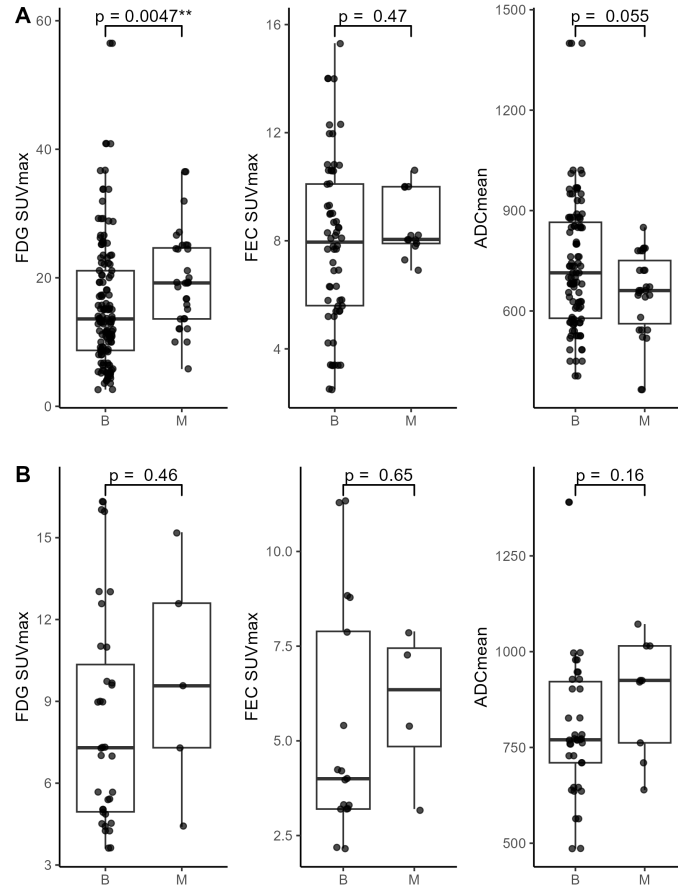


Figure 1: (A) SUVmax was significantly elevated in primary tumours on FDG-PET/CT in patients with malignant (M) lymph nodes compared to those with benign (B) lymph nodes in endometrial cancer. (B) No significant difference in quantitative measures of the primary tumour were found for any imaging modality in cervical cancer. **, $p < 0.01$.